

Clinical SOAP Note Format

Subjective

This section can be seen as the “history” section (i.e., History of Present Illness [HPI]). You can include symptom dimensions, a chronological narrative of the patient’s complaints, information obtained from other sources (always identify the source if it is not the patient), pertinent past medical history, a pertinent review of systems (e.g., “Patient has not had any stiffness or loss of motion of other joints”), and current medications (list with daily dosages).

Objective

This section is the physical exam and laboratory data section. This section focuses on the physical exam and all pertinent labs, X-rays, etc. completed at the visit. This section includes facts that can be verified, such as vital signs, labs, swelling, discoloration, etc. This section can also include outside notes information.

Assessment/Problem List

This section is your assessment of the patient’s problems. For example:

Assessment: A one sentence description of the patient and major problem.

Problem list: A numerical list of problems identified.

All listed problems need to be supported by findings in the subjective and objective areas above. Try to take the assessment of the major problem to the highest level of diagnosis that you can. For example, “low back sprain caused by radiculitis involving the left 5th LS nerve root.” Provide at least 2 differential diagnoses (East Asian diagnosis/Western diagnosis) for the major new problem identified in your note.

Plan

This section is your plan for the patient based on the problems you’ve identified. Develop a diagnostic and treatment plan for each differential diagnosis. Your diagnostic plan may include tests, procedures, other laboratory studies, consultations, etc. Your treatment plan should include patient education, pharmacotherapy (if any), and other therapeutic procedures. You must also address plans for follow-up (next scheduled visit, etc.). Also see the *Bates’ Guide to Physical Examination and History Taking* for excellent examples of complete History and Physical Exam (H&P) and SOAP note formats.

Sample Language:

Acupuncture with points used for treatment listed; Ashi points with or without e-stim

Accessory techniques performed/location of techniques used: tuina, cupping, guasha, if oils or liniments were used, etc.

Number of treatments planned before reevaluation

Education information: qi gong, diet, meditation, sleep suggestions, herbals or supplementation

Please visit [Purdue Online Writing Lab](#) for more information on SOAP Note sections.

Procedure: Intake and Acupuncture treatment	Today's date: 8/14/2020
All correct equipment/supplies are present and ready for use prior to the procedure.	YES
Patient stated name and date of birth with a picture ID.	YES - DOB 6/14/1972 - NAME: Jane Doe
Patient verbally stated the procedure (including the site and side) to be completed.	YES
Informed Consent reviewed and consistent with procedure.	YES

SUBJECTIVE: Jane Doe is a 48-year-old Female referred to XXXX Department by PCP (Dr. Marcy Jones) and a copy of the report will be sent to that provider by electronic medical record.

This patient was sent for evaluation for acupuncture treatments for chronic low back pain and will perform intake face to face and acupuncture treatment #1 under their benefit year effective date Jan 1, 2020.

Ms. Doe reports she has never had acupuncture, or dry needling but does see a chiropractor weekly for her chronic low back pain with only slight benefit to her pain. She says it helps her with mood.

Presenting complaint/Prior Diagnosis: SPINAL STENOSIS OF LUMBAR SPINE, HEADACHES, and ANXIETY

Pain score (1-10) Pain score and area: B-LBP with sciatica, worse on right side, running down right buttock down UB channel to right lateral ankle. Patient reports the pain is 6/10 and jumps up when she first gets up in the morning and has been ongoing since 2018 when she reports she had a bike accident. She reports monthly frontal headaches that come on a day before her menstruation flow starts, but she does not have one today.

Allergies: NKA

Medications: Meloxicam (Mobic) 15 mg 1 tablet PO daily, Pregablin (Lyrica) 50 mg 1 tablet PO daily, Fluoxetine (Prozac) 20 mg PO daily

Heat/Cold	Ms. Doe reports her temperature is about the same as those around her. When she has pain, she uses a heating pad.
Perspiration	She reports she sweats about the same as those around her.
Body pain area	Does patient have any head or body pain? If body pain, what location(s) and what side? B-LBP with sciatica, worse on right side, running down right buttock down UB channel to right lateral ankle. Patient reports the pain is 6/10 and jumps up when she first gets up in the morning. She reports monthly frontal headaches that come on a day before her menstruation flow starts. She reports no abdominal pain or other digestive issues.
Hunger	When it comes to hunger, patient reports that she only eats because she knows she should eat at mealtime.
Thirst	She reports she only drinks because she knows she should.
Temperature of liquids	Her temperature preference for what she drinks is cold (too cold with ice cubes).
Urination	She reports her frequency of urine is more than 5 times a day.
Stool	She reports she has daily to 2 times daily bowel movements, with more formed stools.
Vision	Does the patient wear glasses? If so, how often do they wear them? She reports she wears her glasses all the time.
Hearing	She reports she has ringing in the ears (like a whistle) and has decreased hearing on the right side.
Sleep	How many hours a day does the patient sleep? She reports she has difficulty falling asleep, staying asleep, and difficulty falling back asleep once awoken. She only sleeps a total of 6.5 hours a day, including naps.
Reproductive	When it comes to reproduction/intercourse, is the patient sexually active? Patient reports she is still sexually active, but her partner has medical issues that limit their frequency. She reports still having regular periods. LMP: 7/28/2020
Constitution	She reports she is working as a OR nurse for Nursing temp agency.

Objective:

Vitals: BP:135/77, P:59, Temp 97.9F, Resp: 16, Ht: 5'5", Wt: 108lb 4.8oz, LMP: 7/28/2020

Tongue: slight red tip, slight center crack, thin coat, darker pink tongue

Radial Pulse Right: wiry, slippery, moderate

Radial Pulse Left: wiry, slippery, moderate

Assessment/Problem List:**East Asian Diagnosis (TCM):**

Root: (causes) KD/SP QI def

Branch: (acute issues) wind cold bi

Treatment Strategy: nourish kd/sp qi and release wind cold bi

Plan:

Acupuncture: Treatment with patient facedown (Prone): Right unilateral treatment: UB40, UB57, UB60, KD7, PC6, TW5, UB24, UB23, UB22, ST36, GB34, LI11, LI4, (B)GB20, DU15, ANIMEN,

Ear points (B) Shenman, Liver, Upper Lung, Kidney, Sympathetic

**Needle count**

Seirin
Green 6
Red 6
DBC
18x30 10
Totals 22
Sign: Dr. NCCAOM LAC

Needles removed #: 22

Removed and Needle count verified by staff.

Intake and acupuncture treatment today

Four every other week acupuncture treatments and then reevaluate at 5th visit and make new plan with adjustments if needed.

Education

Diet suggestions given
Exercises given
Neck protection suggested
Qi gong breathing taught

Verbal consent for today's treatment, **sign consent on file. Verbalized understanding of the consent and had an opportunity to have all questions answered. Risks and potential complications related to acupuncture procedures were explained to patient and understanding was verbalized.**

Prior to the treatment, the possible complications of acupuncture were again reviewed as were the areas of the patient's pain and expectations of treatment.

Technique: Area identified. Acupuncture performed in typical manner.

Patient tolerated procedure well without difficulty. The treatment time was a 20 minute discussion/education and 20 minutes of needles being placed.

Pain score (1-10) Post treatment: Ms. Doe reports, "Wow! My pain is gone now!" and she will observe for pain relief score and will report back at next visit.

Dr. NCCAOM LAC
Clinic Address Phone

Resources

- Baird, Brian N. (2014). The internship, practicum, and field placement handbook. 7th Ed. New York: Routledge.
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- Moline, Mary E., & Borcharding, Sherry. (2013). The OTA's guide to documentation: Writing SOAP notes. 3rd Ed. Thornfield, NJ: Slack Inc.
- Moline, Mary E., Williams, George T., & Austin, Kenneth M. (1998). Documenting psychotherapy: Essentials for mental health practitioners. Thousand Oaks, CA: Sage.
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https://owl.purdue.edu/owl/subject_specific_writing/healthcare_writing/soap_notes/major_sections.html
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